

CLINICAL PRACTICE

Telepsychology Consent & Multi-State Practice Checklist

A patient consent addendum and a clinician decision flow for one question that matters every session: where is the patient, and am I allowed to treat them there?

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Consent + decision flow

VERSION

1.0 · Sept 2026

TIME TO ADAPT

30–45 minutes

WHAT IT IS

A telehealth consent addendum for patients, plus a decision flow and checklist for clinicians who treat patients across state lines.

USE IT WHEN

You provide any care by video or phone, or a patient may travel, move or be located in another state.

CUSTOMIZE FIRST

Your license states, PSYPACT status, telehealth platform, emergency procedures and your payers' telehealth rules.

The question that governs everything

For telehealth, the legal question is usually where the patient is physically located at the time of the session, not where they live and not where you sit. Providing services to a patient in a state where you are not licensed or otherwise authorized is unlicensed practice. Part B gives you a repeatable way to answer.

PSYPACT IN ONE PARAGRAPH

The Psychology Interjurisdictional Compact lets eligible psychologists practice telepsychology across participating jurisdictions without a full license in each. You need an E.Passport from ASPPB and an Authority to Practice Interjurisdictional Telepsychology (APIT) from the PSYPACT Commission. For APIT the psychologist generally must be licensed in, and physically located in, their declared PSYPACT home state while providing services. A separate Temporary Authorization to Practice (TAP) permits limited in-person practice in participating states, typically up to 30 days per calendar year. Participation changes as states join, so verify current membership at psypact.gov and do not rely on a list you saved.

Part A: Telepsychology consent addendum (patient)

Telehealth consent for [Patient name]. I agree to receive some or all of my care from [Clinician name] by video or phone ("telehealth").

What telehealth is. Telehealth means providing care over secure video or phone instead of in person. I understand it can make care easier to reach, and that it has limits: some concerns may be better treated in person, and technology can fail.

My location. I will tell [Clinician] where I am at the start of every session, including my address, and I will let them know before I travel or move. [Clinician] can provide services only when I am in a state where they are licensed or otherwise legally authorized. If I am somewhere they are not authorized, we will reschedule or arrange another option.

Privacy. I will join from a private place, use headphones if needed, and make sure no one else can overhear unless I agree. I will not record sessions, and [Clinician] will not record them, unless we both agree in writing. I understand that technology carries privacy risks even when the platform is secure. I will keep my device secure and avoid public Wi-Fi.

Emergencies. Before we begin, I will give [Clinician] my current address and an emergency contact: [name, phone]. If I am at risk and cannot be reached, [Clinician] may contact emergency services or my emergency contact. I will call 911 in an emergency and can call or text 988 in crisis.

If the connection fails. [Clinician] will call me at [my phone] within [five] minutes. If we cannot reconnect, we will reschedule and I will not be charged.

Fees and insurance. My plan may or may not cover telehealth. I am responsible for any costs my plan does not cover.

Right to stop. I can stop telehealth at any time and ask to meet in person if available. I can ask questions about telehealth at any time.

Patient signature

Date

Clinician signature

Date

Part B: Where can I lawfully treat this patient?

Work through these steps at intake, before travel, and whenever a patient says they are moving or away. Record the answer.

1. **Locate.** Where will the patient physically be at the time of each session? Ask at every session for telehealth patients with any history of travel or a second home.
2. **Licensed there?** Do you hold a full license in that state? If yes, proceed.
3. **Compact route.** If not, does the patient's location participate in PSYPACT, and do you hold a current E.Passport and APIT? Is your home state a participating state, and are you physically located in it during the session?
4. **Other authority.** Some states allow short-term, continuity-of-care, or emergency exceptions. Rules vary and change. Check the patient's state board in writing before relying on one.
5. **Payer rules.** Does the payer cover telehealth, from what location, with which place-of-service code and modifier? Does the plan require a state license or network status in the patient's location?
6. **Prescribers.** For prescribing: are you licensed in the patient's state, and are you complying with DEA and state telemedicine rules for controlled substances? The DEA telemedicine flexibilities were extended through December 31, 2026; check the current status (see PA-14).
7. **Document.** Record the patient's location, your authority, and the source you checked.
8. **Plan for movement.** Agree in advance what happens if the patient travels: a pause, a session by phone only if lawful, or a transition to a local clinician.

Location and authority log

Date	Patient location (state)	Authority (license / PSYPACT / other)	Checked source	Notes

Pre-session checklist

- ☐ Confirm patient identity and current physical address.
- ☐ Confirm emergency contact and nearest emergency number for their location.
- ☐ Confirm they are alone or who else is present, and that they are in a private place.
- ☐ Confirm the platform is HIPAA-compliant, covered by a BAA, and updated.
- ☐ Confirm the patient's phone number and your backup plan if video fails.
- ☐ For higher-risk patients, confirm access to a safety plan and a person nearby.

Verify these at the source

What	Where to check
PSYPACT participating jurisdictions, APIT and TAP	psypact.gov
E.Passport, IPC and credentials	ASPPB (asppb.net)
State license status and rules on telehealth	The patient's state psychology or medical board
Physician interstate licensure (psychiatrists)	Interstate Medical Licensure Compact and each state medical board
Controlled-substance telemedicine rules	DEA and your state board; see PA-14
Payer telehealth rules	Each payer's provider manual

ANNUAL RENEWALS

PSYPACT authorizations must be renewed each year, and the E.Passport renewal requires technology-related continuing education. Track the renewal dates in your compliance calendar (PA-19).

IN PSYALLIANCE

Add your PSYPACT authority in your profile and it will appear as a badge in Network and Referral matching. PsyAlliance does not maintain a state list, so always confirm current membership at psypact.gov.

Sources and further reading

- PSYPACT Commission (psypact.gov): APIT, TAP and E.Passport information.
- ASPPB, Practicing Telepsychology Under PSYPACT (asppb.net).
- APA, Guidelines for the Practice of Telepsychology (check apa.org for the current revision).
- DEA and HHS, Fourth Temporary Extension of COVID-19 Telemedicine Flexibilities (Federal Register, 31 Dec 2025).

ABOUT THIS DOCUMENT

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